

TITLE: Exposure Keratopathy Prevention in the ICU

PROTOCOL: Exposure Keratopathy Prevention

PURPOSE: Exposure keratopathy is a common but preventable complication in the ICU that up to 42% of all ICU patients and 56% of all mechanically ventilated patients may develop. (1)
The implementation of a monitoring and prevention protocol substantially reduces overall rates of EK to as low as 2.6%. (2)

APPROVED:



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DEFINITIONS:

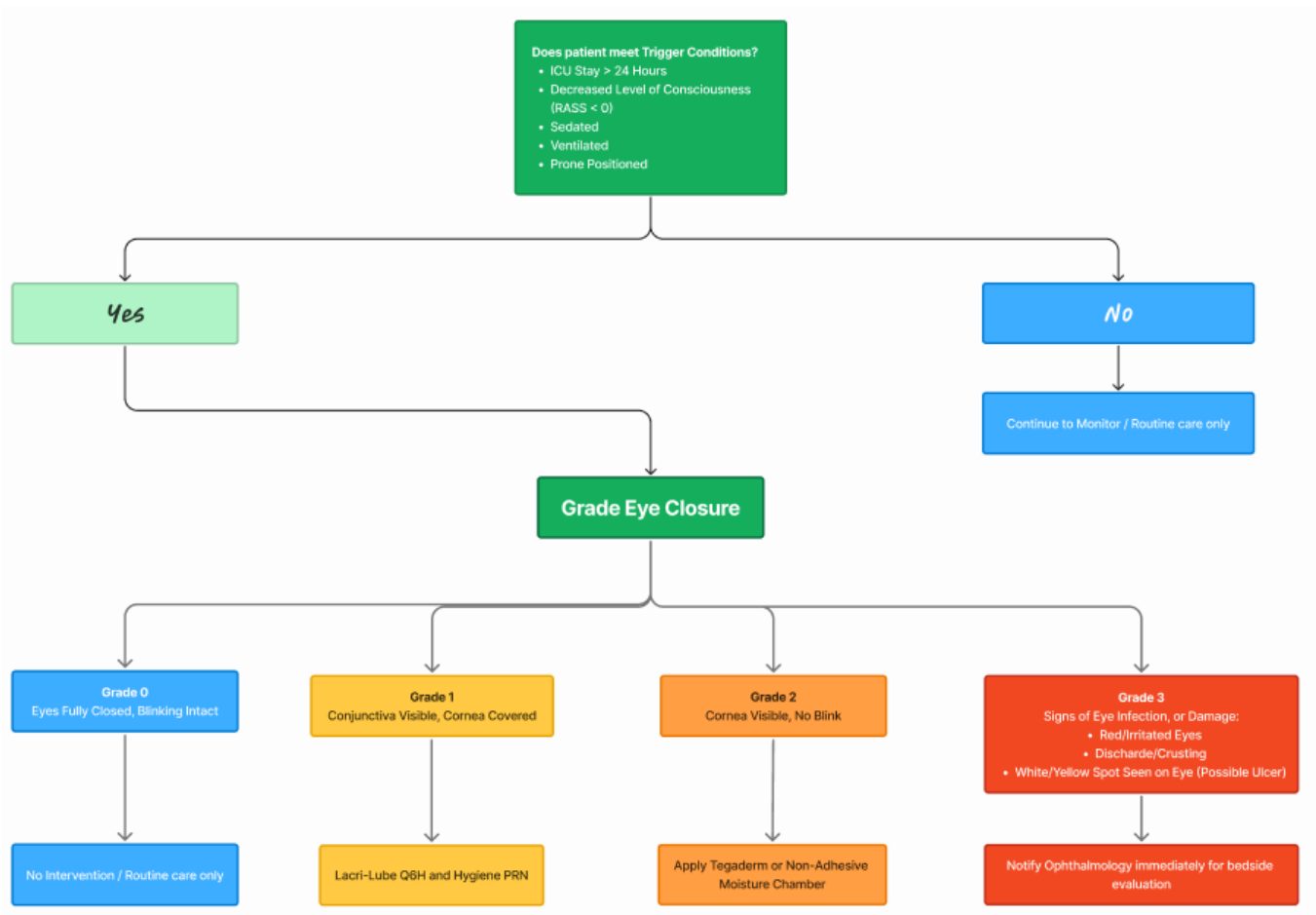
- A. RASS – Richmond Agitation Severity Scale
- B. Q6H – Every 6 Hours
- C. Q12H – Every 12 Hours
- D. PRN – As Needed
- E. EK – exposure keratopathy

CORE PROCEDURE:

1. Eye Risk Screening – to be completed Q12H
 - a. Trigger Conditions
 - i. ICU stay > 24 hours
 - ii. Decreased Level of Consciousness (RASS < 0)
 - iii. Sedated
 - iv. Ventilated
 - v. Prone Positioning
 - b. Evaluation of Eye Grading
 - i. Grade 0 – eyelids fully closed, blinking intact
 - ii. Grade 1 – Conjunctiva visible, cornea still covered
 - iii. Grade 2 – Corneal visible, no blink
 - iv. Grade 3 – Signs of Eye Infection or Damage, such as red/irritated eyes, eye discharge/crusting, white/yellow spot seen on eye (possible ulcer)
2. Eye Protection Interventions
 - a. Grade 0 – No intervention / Routine Care only
 - b. Grade 1 – Lacri-Lube Q6H and Hygiene PRN
 - c. Grade 2 – Apply Tegaderm or Non-Adhesive Moisture Chamber
 - d. Grade 3 – Notify Ophthalmology immediately for bedside evaluation

APPENDIX

Appendix A:



KEYWORDS:

Exposure Keratopathy, Corneal Ulcer, Lagophthalmos

REFERENCES:

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Kousha O, Kousha Z, Paddle J. Incidence, risk factors and impact of protocolised care on exposure keratopathy in critically ill adults: a two-phase prospective cohort study. Crit Care. 2018 Jan 16;22(1):5. doi: 10.1186/s13054-017-1925-5. PMID: 29338772; PMCID: PMC5771067.